Royal Devon University Healthcare NHS Foundation Trust

Rapid Tranquilisation (RT) Policy Inpatient acute ward-based areas for adults 18yrs >	
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Eastern 1.0 RT guideline	17.12.15	Jointly with Consultant Nurse Older People and Consultant Physician	1st RT guideline for RD&E Trust
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In consultation with and date:
Email sent to the following teams 18.5.23 for their review of merged document;

- Chief Medical Officer, Chief Nurse, Site Medical Directors & Directors of Nursing (East & North)
- Dementia and Delirium Steering Group
- Dementia & Delirium specialist nurses / Admiral Nurses Eastern and Northern
- Consultant Geriatricians and Registrar Healthcare for Older People Department (East)
- ADoN's (East & North)
- Senior Nurses Northern
- ED & AMU Physician's and senior nurses (East & North)
- Ward managers Eastern and Northern
- Paediatric ADoN and Senior Seniors (East & North)
- Learning Disability Teams (East & North)
- DPT psychiatric Liaison team (East & North)
- Pharmacy (East & north)
- Drugs & Therapeutics Committee
- Executive team
- Governance Divisional Teams (East & North)
- Site teams (East & North)
- Security Teams (East & North)
- Safeguarding & MCA Team (East & North)

Equality impact assessment reviewed by Andrea Bell 7.9.23

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CONTENTS

RAPID TRANQUILISATION (RT) FLOWCHART – Lorazepam example 4
TRUST STATEMENT ON RESTRAINT AND RESTRICT PRACTICE 5
IMPORTANT NOTE ON POLICY EXCLUSIONS 5
1 INTRODUCTION 5
2 SPECIAL NOTE - IM antipsychotic / benzodiazepine sedation when oral route is not available 6
3. PURPOSE 6
4. DEFINITIONS 7
5. DUTIES AND RESPONSIBILITIES OF STAFF 8
6. PATIENT SAFETY CONSIDERATIONS 10
7. ROLE OF SECURITY STAFF 11
8. PATIENT PRIVACY AND FAMILY INVOLVEMENT 11
9. DRUG ADMINISTRATION CONSIDERATIONS 12
10. PHYSICALLY FRAIL PEOPLE OR THOSE WITH DEMENTIA (any age) 12
11. LEARNING DISABILITIES 13
12. YOUNG ADULTS 16-18yrs 13
13. CIRCUMSTANCES WHERE CAUTION IS RECOMMENDED 13
14. RT IN PREGNANCY 14
15. ASSESSMENT, PRESCRIPTION AND ADMINISTRATION 14
16. CHOICE OF MEDICATION 15
17. REQUIRED MONITORING POST-ADMINISTRATION OF RT 15
18. POST INCIDENT REVIEW AND MONITORING REQUIRED AFTER RT 16
19. LEGAL IMPLICATIONS FOR RT 17
20. MONITORING COMPLIANCE WITH THIS GUIDELINE 18
21. ASSOCIATED CLINICAL GUIDELINES OR POLICIES/PROCEDURES 18
22. ARCHIVING ARRANGEMENTS 19
23. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY/ STRATEGY 19
APPENDIX 1: EPIC RAPID TRANQUILISATION (RT) FLOWSHEET 20
APPENDIX 2: EPIC RAPID TRANQUILISATION ORDER SETS 21
APPENDIX 3: EPIC RT REVIEW AND WORK LIST ACTIVATION 23
APPENDIX 4: MANAGEMENT OF CLINICAL CONCERNS DURING RT 24
APPENDIX 5: DRUG TREATMENTS FOR RAPID TRANQUILLISATION (RT) 25
APPENDIX 6: HOW TO USE FLUMAZENIL 26
APPENDIX 7: DRUG INFORMATION FOR DRUGS INCLUDED IN RT POLICY 27
APPENDIX 8 : STRATEGIES TO MANAGE CHALLENGING BEHAVIOUR 31
APPENDIX 9: COMMUNICATION PLAN 32
APPENDIX 10: EQUALITY IMPACT ASSESSMENT TOOL 33

RAPID TRANQUILISATION (RT) FLOWCHART – Lorazepam example

Check for reversible medical issues causing distress ie pain, retention, hypoxia etc.
Always continue non-pharmacological techniques ie de-escalation, prevention & distraction (see also Delirium Guidelines).
Consider oral lorazepam or olanzapine prior to considering Intramuscular (IM) Rapid Tranquilisation
IM Rapid Tranquilisation may be the first thing you think of, but should, in most cases, be the last thing you do

- Only if advice above has been ineffective, and the patient remains a harm to themselves or others, should you consider IM Rapid Tranquilisation (RT)
- A mental capacity assessment will be required and documented. Only if the patient lacks capacity to keep themselves or others safe, should RT be considered
- Lorazepam should always be the first line drug of choice (see appendix 5 for drug choices)

Under 65ys
(with no dementia)

Offer oral drug treatment
Lorazepam 1-2mgs
Max 4mgs /24hrs

Over 65yrs
(or any age with dementia / low body weight)

Offer oral drug treatment
Lorazepam 0.5-1mgs
Max 2mgs /24hrs

Observe for 30-60 mins, if no or limited response and the patient is still a harm to themselves or others;

- Continue de-escalation, distraction and prevention ie are they in pain, hypoxic?
- Consider repeating oral medication dose if safe to do so
- If situation continues to escalate and the patient / others are at risk of harm, prepare for IM Rapid Tranquilisation
- Call for help from medical team, senior nurse, site manager, security, as appropriate
- Intravenous (IV) Rapid Tranquilisation for RT should **NEVER** be administered on a ward without an anaesthetist present – high risk of respiratory arrest and sudden death

Intramuscular treatment (IM)
Lorazepam IM 2mgs
Max 4mgs /24hrs from all routes
Check flumazenil available for reversal

Wait 60 mins before considering a 2nd IM dose of RT
Follow RT flowsheet for observations
Risk of death and adverse complications following RT , monitor closely
Document in notes and Datix event

Activate in EPIC:

RT order set for drug prescription
RT flowsheet for after care guidance

Intramuscular treatment (IM)
Lorazepam IM 1mgs
Max 2mgs /24hrs from all routes
Check flumazenil available for reversal

Wait 60 mins before considering a 2nd IM dose of RT
Follow RT flowsheet for observations
Risk of death following RT, monitor closely
Note ; this patient group most at risk of adverse complications due to prolonged sedation effects leading to adverse events ie dehydration, pressure damage, chest infection, falls
Document in notes and Datix event

TRUST STATEMENT ON RESTRAINT AND RESTRICT PRACTICE

Royal Devon University Healthcare NHS Foundation Trust (RDUH), here after referred to as the Trust, is committed to delivering the highest standards of health, safety and welfare to its patients, visitors and employees.

The Trust believes that the management of challenging behaviour is an activity requiring humanity and respect for the rights of the individual, balanced against the risk of harm to themselves, staff and members of the public. Restricting or restraining any behaviour should only be used when it is proportionate and reasonable to do so.

It is recognised that violent and aggressive behaviour can escalate to the point where physical or chemical restraint may be needed to protect the person, staff or other legitimate users of Trust premises and facilities from significant injury or harm, even if all best practice to prevent such escalation is deployed. Restraint is a last resort intervention and will only be considered when all other practical means of managing the situation, such as de-escalation, involvement of family where appropriate, verbal persuasion, voluntary 'time out', or gaining consent to taking medication, have failed or are judged likely to fail in the circumstances.

The self-respect, dignity, privacy, cultural values, race and any special needs of the patient should be considered in so far as is reasonably practicable. In the event of the application of physical or urgent chemical restraint an Incident Form must be completed. Used inappropriately, some restraint may be classed as a form of abuse or assault and if unreasonable may lead to investigation. If restraint of any kind is used it should be carefully recorded in the patient records and monitored.

IMPORTANT NOTE ON POLICY EXCLUSIONS

This policy is for inpatient acute ward-based areas. It **DOES NOT** cover paediatric areas, community inpatient wards, Intensive Care Units or Emergency Departments. These areas are expected to have their own Standard Operating Procedure (SOP) for Rapid Tranquilisation events. The SOP's for these areas should be based on the fundamental principles of this policy, but directed to the specific needs of their clinical environments and patient groups.

1. INTRODUCTION

1.1 This document sets out the Royal Devon Healthcare University Trust (RDUH) requirements for Rapid Tranquilisation (RT) for inpatient acute ward areas

1.2 The use of an injectable medication to manage acutely disturbed and potentially dangerous behaviours is only one therapeutic approach to care and should not be first line management

1.3 Staff should make every effort to assess any medical or environmental causes of distress, whilst also using a range of de-escalation techniques such as moving the patient to a low stimulus area before considering using a chemical restraint

1.4 Urgent referral to specialist teams such as Dementia & Delirium nurse specialist, Mental Capacity Assessment team (MCA), Psychiatric liaison or learning disability team, should be considered at an early stage for advice and support

1.5 RT procedure **MUST** only be considered for a patient who;

- lacks capacity
- are displaying acutely disturbed and potentially dangerous behaviours that may be a harm to themselves or others
- have not responded to verbal de-escalation and other non-pharmacological interventions first

1.6 Caution should be taken with older patients, particularly those with organic brain disease as they may be particularly sensitive to side effects of medications.

1.6.1 Patients with Parkinsonian syndromes, including Lewy Body Dementia (LBD), can suffer marked reactions to anti-psychotic medications.

1.6.2 They have severe neuroleptic sensitivity with increased risk of permanent damage and death, over 80% LBD have a reaction, 50% severe

1.7 In addition to this policy, staff should be familiar with related Trust policies including;

- Delirium Guideline
- Consent policy
- Medicines policy
- Mental Capacity Act Policy
- Restraint and Restrictive Practice Policy
- Resuscitation Policy
- Enhanced Care Policy (North)

2 SPECIAL NOTE - IM antipsychotic / benzodiazepine sedation when oral route is not available

2.1 If an intramuscular (IM) antipsychotic or benzodiazepine is prescribed as an alternative route of administration, where an individual is refusing the oral drug and require it for their standard medical treatment ie Behavioural & Psychological Symptoms of Dementia (BPSD), or they cannot absorb through an oral route ie upper GI /colorectal complications, but they are otherwise not acutely disturbed, then this is **NOT** rapid tranquillisation.

2.2 The drug in this case should be prescribed and clearly marked that it is not to be used as part of rapid tranquillisation (RT) – there is no need for a Datix report to be completed.

2.3 However, the patient should still be monitored carefully following IM sedation as they could still deteriorate post administration.

2.4 Legal authority for administering IM sedation should also be considered.

2.5 It should be noted that for many drugs, this would be an unlicensed indication. If there is any doubt whether the prescription falls within the criteria of Rapid Tranquillisation, then it must be treated as rapid tranquillisation and this protocol followed fully.

3. PURPOSE

3.1 To provide practice standards to all staff who are involved in the prescription or administration of IM Rapid Tranquilisation medication for the control of disturbed behaviour

3.2 The aim of RT intervention is an emergency measure to quickly calm a person and reduce the risk of further violence and harm to themselves or others, rather than treat any underlying mental illness

3.3 The objective is not to induce sleep or unconsciousness, but to promote a calmer state where the individual is still able to participate in further assessment and/or treatment, and retain their dignity and safety if their challenging behaviours are placing this at risk

3.4 Rapid tranquilisation should only be given by or under the direct supervision of Registered medical/ nursing/ ACP/ ODP staff, with access to appropriate monitoring, rescue drugs, equipment for managing airway and circulatory emergencies

3.5 Rapid tranquillisation with injectable medication (IM) should be considered in the following circumstances, where oral therapy and non-pharmacological interventions have not been effective and a person with disturbed mental functioning who lacks capacity:

- Is exhibiting behaviour that is destructive, dangerous or aggressive
- Is at risk of exhaustion
- Needs rapid calming to alleviate immediate and severe distress

4. DEFINITIONS

4.1 **Rapid Tranquillisation (RT):** refers to the use of injectable intramuscular (IM) medication to manage violent or aggressive behaviour safely and effectively, or to manage behaviour in individuals, which if it does not stop, it is thought will result in harm to the individual or others.

4.1.1 The aim of the intervention is an emergency measure to quickly calm an individual down and reduce the risk of further violence and harm to themselves or others, rather than treat any underlying mental illness

4.1.2 This procedure only covers the use of injectable IM medication for RT

4.1.3 Other interventions including verbal de-escalation and oral medication are beyond the scope of this procedure but must be considered prior to proceeding with injectable medication (See Delirium Guideline on HUB)

4.2 **Restrictive interventions:** are a specific subset of restrictive practices. They are deliberate acts on the part of the other person(s) that restrict a patient's movement, liberty and/or freedom to act independently to;

- Take immediate control of a dangerous situation where there is a real possibility of harm to the person or others if no action is taken
- End of significantly reduce the danger to the patient and others

Examples of restrictive interventions that may be used are;

- Physical intervention and restraint (including mechanical)
- Rapid Tranquilisation
- Personal and other searches
- Enhanced Observations
- Withholding equipment or information
- Blanket restrictions

4.3 **Physically frail adults**: This will include; older adults over 65 years, those with a multimorbid medical condition, adults who are underweight, adults of any age who have dementia (particularly Lewy Body), adults at the end of life or currently reside in a care home

4.4 **Lacks capacity**: A legal term used in individuals aged 16 years and over who may lack the mental capacity to make their own decisions about their care and treatment. Capacity

assessments are time and decision specific and must be in accordance with the legal test of capacity (The Mental Capacity Act 2005, Code of Practice Chapter 4)

4.5 **Best Interest:** The best interest principle in the Mental Capacity Act 2005 states that any act done or decision made on behalf of an adult lacking capacity must be in their best interest

4.5.1 The decision maker must have objective reasons for any decision they make
4.5.2 They must be able to show that they have considered all circumstances relevant to the decision in question
4.5.3 Actions taken on behalf of the patient who lacks capacity must be proportionate and least restrictive. (The Mental Capacity Act 2005, Code of Practice Chapter 5)

4.6 **De-escalation**: A range of calming verbal and physical techniques designed to calm the patient without force

4.7 **Datix**: The Trust's risk management system (see Incident Reporting, Analysing, Investigating and Learning Policy and Procedures.)

4.8 **RIDDOR:** Reporting of Injuries, Diseases, Dangerous Occurrences Regulations 1995 requires certain types of incidents to be reported to the Health and Safety Executive. This includes accidents which result in a person not at work (e.g. a patient, visitor) suffering an injury and being taken to a hospital, or if the accident happens at a hospital, suffering a major injury which would otherwise have required hospital treatment that is caused by work activity or failure of equipment. This does not include incidents caused by the clinical condition which were being managed appropriately as part of a risk assessment or care plan

4.9 **A RT episode** - is defined as the time from which RT is administered until the situation has resolved and the individual requires no further intervention at this time

4.10 **Deprivation of Liberty (DoLS) -** A legal framework which, when in place, allows a patient to be restricted to a specific place for the purposes of care and treatment. If the patient requires specific care of treatment ie Rapid Tranquilisation, mittens to stop pulling out an NG tube etc then a separate capacity assessment will be required for this specific treatment at that time

5. DUTIES AND RESPONSIBILITIES OF STAFF

5.1 **The Board of Directors** are responsible for:

- An overarching approach for the Trust's zero tolerance to harm
- Ensuring policies and procedures are in place to prevent and minimise the use of restrictive practices, including Rapid Tranquilisation
- Ensuring that adequate resources are made available to: monitor incidence and prevalence of RT and associated harm, to provide education and training and to provide suitable and sufficient equipment
- Ensuring that there is a process in place for reporting and learning from Serious Incidents Requiring Investigation (SIRI)
- Ensuring training in Rapid Tranquilisation, control & restraint and de-escalation techniques are available for all staff who require it

5.2 **The Trust Directors of Nursing** has overall responsibility, on behalf of the Board, for ensuring the implementation of this policy throughout the organisation

5.3 **The Assistant Directors of Nursing** are responsible for ensuring that the processes outlined in this policy are implemented within their Division

5.4 **The Associate Director for Safety and Quality** in conjunction with the **Safety and Risk Team** and **Divisional Governance Teams** are responsible for:

- Reviewing all relevant incident forms reported via Datix
- Identifying when a RIDDOR investigation is required and providing expert assistance to the local investigation team
- Ensuring organisational systems are in place for the governance of clinical procedures

5.5 **All staff prescribing and / or administering rapid tranquillisation i.e Doctors, Non-Medical Prescribers (NMP), ACP's, Registered Nurses** etc**,** have a responsibility to ensure that:

- They complete RT training and updates
- They follow Trust RT policy
- Have completed the appropriate training for the safe and appropriate restraint of a patient to include conflict resolution and safe handling
- Prescribe RT medications correctly using the EPIC RT order set to guide their practice for the correct age group and underlying condition (Appendix 2)
- The RT flowsheet in EPIC is activated once RT has been administered to ensure the patient is observed for signs of deterioration post procedure (Appendix 1 & 3)
- The RT event is clearly documented in the EPIC record and updated on the e-discharge summary to hand over to community services
- A Datix report is completed each time RT is administered
- **Rapid Tranquilisation is NOT in the scope of the Nursing Associates role**

5.6 **Consultants with trainee Doctor responsibility / Clinical Matrons / Clinical Nurse Managers** (eastern) / **Ward Managers** (Northern) have a responsibility to ensure that:

- Staff have access to appropriate restraint and conflict resolution training, to include safe holding training to enable safely administer RT, as outlined in this procedure. Working to the principles of least restrictive intervention for least amount of time to achieve the required outcome
- Staff have access to training in RT administration for those staff who may be required to administer RT medication or care for a patient post procedure
- Appropriate medical equipment is available on the ward to allow patient monitoring where RT is administered (i.e. pulse oximeter)
- Appropriate medication for RT and for the emergency management of side effects of RT is available on the ward (i.e. flumazenil – see appendix 6)
- A Datix is completed every time a patient is given RT
- Staff are aware that capacity and best interest assessments are documented which describe the decision making and care planning in regard to RT
- Families/ carers are informed about an RT event and the requirement for DoLS
- Encourage staff de-briefs following a serious clinical incident, and offer staff support from Occupational Health team, or they can self-refer

5.7 **Site Management Teams**

- Site Management Team to have an overview, in conjunction with other stakeholders, of patients that are posing a management challenge and may or have required RT

- Where required, to attend and support both security and ward teams in the de-escalation phase and of an incident and support with safe rapid tranquillisation
- Where ward teams or response teams are struggling to navigate an urgent situation to support the situation to safe resolution
- All Clinical Site Practitioners to be up to date on relevant training including de-escalation and breakaway, advanced physical interventions, mental capacity assessments and all managers safeguarding training
- To escalate specific concerns to the relevant cluster management team in office hours if not already aware and to the on-call clinical matron and manager out of hours
- Sit management teams should be contacted through their usual referral systems for each hospital

5.8 **Support teams** ie Dementia & Delirium specialist nurses, Learning Disability team, MCA team, psychiatric liaison

- The role of specialist teams may be helpful as part of the de-escalation period prior to the need for RT and as such the most appropriate team should be asked to advise and help as appropriate
- Specialist teams should be contacted through their usual referral systems for each hospital
- These teams will document their involvement in the patient's notes directly and are encouraged to using the Datix system to record any associated concerns with the RT event

5.9 **All staff**

- Being involved in a RT situation can be very distressing and traumatising for staff.
- Staff are encouraged to seek out support from their peers and senior staff on the day
- They are also encouraged to talk it through with their line managers, clinical supervisors etc
- If staff require additional support they can ask their manager for a referral to occupational health or they can self-refer themselves

6. PATIENT SAFETY CONSIDERATIONS

6.1 Only proceed with RT if verbal de-escalation and other interventions (eg pain relief if appropriate) have been ineffective and the behaviour is still a risk to self or others

6.2 RT must only be employed when it has been established that the risk of not doing so is greater than the risk of acute pharmacological treatment

6.3 Caution must always be exercised as there are serious adverse reactions associated with RT, which may result in fatality, especially where the individual is unknown to the services and their reaction to medication is not predictable

6.4 Where RT is appropriate, non-pharmacological strategies i.e de-escalation and distraction, should be deployed before considering RT and then in collaboration with RT event once given. The combined use of multiple strategies is likely to augment the effect and desired outcome

6.5 All medication used in rapid tranquillisation must be given time to work before further medication is administered. A minimum of 60 minutes is recommended to avoid risk of respiratory depression

6.6 NICE Clinical Guidelines (Violence NG10) recommends when RT is administered, and particularly where restrictive interventions may be used, there should be staff trained in basic life support, with resuscitation equipment immediately available. The following actions must be adhered to:

6.6.1 RT can only be authorised and prescribed by a doctor or an experienced Non-Medical Prescriber (NMP) who has knowledge in this area of care

6.6.2 If the patient rapidly deteriorates following the administration of RT, then the patient's medical team should be contacted urgently, or the on call medical team to assess the person. In addition, emergency response teams are also available on Ext 2222

7. ROLE OF SECURITY STAFF

7.1 Clinical staff requesting help from the security team need to clearly inform the security staff on their arrival why they are requesting their help using the SBAR model. An example might be, 'Rapid Tranquillisation is in the best interest of the patient as they lack capacity to protect themselves'

7.2 The security team cannot be expected to restrain a patient just because a member of staff tells them to do so. There must good, sound clinical reasons to do so and the clinical staff must continue to care for the patient through the incident to monitor the patient

7.3 Clinical staff must document clearly in the patient notes that security was called and the reasons i.e. best interest of the patient who lacked capacity'. This should include details of the patient's behaviour prior to RT

7.4 Security staff should work in conjunction with clinical staff and take advice regarding the appropriate management of the patient which is likely to vary according to the age and cognitive function of the patient

7.5 An incident form on Datix must be completed by either the clinical staff on the ward or the security team after the event

8. PATIENT PRIVACY AND FAMILY INVOLVEMENT

8.1 It is recognised that administration of RT may be distressing to relatives and other ward patients and visitors. When safe to do so every effort should be made to ensure privacy and dignity of the individual as outlined above

8.2 It is accepted, that there will be circumstances which will necessitate the restraint and administration of RT to an individual in areas which are not private. Such circumstances include:

- Where during the course of negotiation, an individual becomes aggressive and immediately attempts to attack others
- Where the level of resistance and aggression from an individual has reached a threshold that meant relocation to a private area would involve unnecessary risk

8.3 The decision to RT the patient is often made quickly with little time to call the family or carer about what is happening. However, as part of the de-escalation techniques the family/carer should be called and informed that their family member is distressed and asked if they are able to offer any help to calm them down

8.4 The involvement of the family might avoid the need to proceed to RT and should always be considered. Where this is not possible, every attempt must be made to inform the family/carer that RT has been administered and the reasons for this

8.5 Consideration needs to be made to keep the family member safe at all times. If the situation is deemed too serious for anyone to be left alone with the patient, then family members should always be accompanied

9. DRUG ADMINISTRATION CONSIDERATIONS

9.1 All medication used in RT must be given time to work before further medication is administered. A minimum of 60 mins is recommended. Longer intervals are needed for IM olanzapine (2 hours)

9.2 Intravenous (IV) administration of RT is **NOT** to be administered in a ward setting, due to the high risk of respiratory arrest and instant death, **UNLESS** an anaesthetist is present ie Arrest Team/ Medical Emergency Team (MET) Ext 2222. It should only be considered as a last resort when the situation is very dangerous to all parties. It is acknowledged that in the Emergency Department, it may be necessary to administer IV sedation for an RT event as there is full resuscitation capability in the event of respiratory distress.

9.3 Violent behaviour can usually be controlled and managed without the need to use high doses or 'cocktails' of medication. The aim of RT is to use the **lowest effective dose** to promote calmness and reduce the risk of further violence. It is recognised that it is sometimes necessary to exceed the BNF maximum recommended doses. In these situations, a Consultant or Registrar/ senior grade Dr, should make that decision and the reasons for exceeding doses must be documented

10. PHYSICALLY FRAIL PEOPLE OR THOSE WITH DEMENTIA (any age)

10.1 General considerations for those who are physically frail:

10.1.1 'Start low & go slow' with respect to doses of medication prescribed

10.1.2 Absorption and onset of action may be delayed- this must be taken into consideration before administering further doses if initial response to treatment poor

10.1.3 Older people may have a larger volume of distribution, leading to longer duration of action and possible accumulation of medication, therefore increasing risk and duration of adverse effects experienced

10.2 Concurrent physical illness and/ or conditions known to alter drug metabolism:

- Renal or hepatic impairment.
- Organic disorder/ delirium.
- Alcohol or drug intoxication.

10.3 Consult the current edition of the BNF and / Summary of Product Characteristics for the product to obtain further information

10.4 The use of RT in people who are physically frail should be rare, but it is recognised that there are specific circumstances where its use will be considered appropriate

10.5 The decision to use RT in physically frail individuals must always be made jointly by the multi-disciplinary team (MDT), and wherever possible clinical staff involved should seek peer support

from staff working in older persons care with more experience in the use of this intervention. This could include Dementia & Delirium/ Admiral specialist nurses, Psychiatric Liaison team (see HUB or call switchboard for North & East contacts)

10.6 Physically frail individuals are more likely to have concurrent medical conditions (such as cardiovascular, respiratory and/ or neurological disorders), and are therefore potentially at a greater risk of experiencing adverse effects of the medication used for RT (i.e. antipsychotics are known to increase the risk of Cerebral Vascular Event in individuals with a diagnosis of dementia)

10.7 RT is **NOT** appropriate for the management of behavioural and psychological symptoms of dementia, unless presentation is acute physically violent/aggressive behaviour with imminent risk of harm

11. LEARNING DISABILITIES

11.1 People with a learning disability may carry an increased risk of certain health complications such as cardiac and respiratory disorders, which contribute to the potential hazards associated with restraint and the administration of medication for RT

11.2 People with a learning disability experience pain the same as anyone else, but may communicate it in a different way. It is important that pain is considered as a possible cause for a change in a person's behaviour

11.3 Where RT is being considered for a patient who has learning disability, the LD team should be informed and asked to assist with advice regarding de-escalation and calming measures. See HUB or call switchboard for North & East contact details

12. YOUNG ADULTS 16-18yrs

12.1 This policy no longer covers young adults 16-18yrs. A new merged policy is being developed which should be referred to for this age group when available

12.2 While the new merged paediatric RT policy is being written, staff in the North should refer to the '*Assessment & Management of Children & Young People with Mental Health Needs Admitted to Hospital Standard Operating Procedure'* and staff in the East should refer to the *Clinical Guideline for: Pharmacological Management of Disturbed Behaviour in Young People, including Rapid Tranquillisation'.*

12.3 It should be noted that if a young person >16yrs up to 18yrs is nursed in an adult area and requires RT, the adult nurse should consider activating the paediatric RT flowsheet, instead of the adult RT flowsheet. Professional discretion can be considered based on height and weight of the young person.

13. CIRCUMSTANCES WHERE CAUTION IS RECOMMENDED

13.1 Each patient should be assessed for their own risks, but specific concerns are:

13.1.1 Congenital prolonged QTc syndromes present (e.g. Romano-Ward Syndrome or Lange-Nielsen Syndrome)

13.1.2 Concurrent prescription or use of other medication that lengthens QTc intervals both directly and indirectly (e.g. QTc >0.45 seconds in men and > 0.47 seconds in women). Many psychotropic drugs affect the QTc interval, including antipsychotics, tricyclic antidepressants and lithium. This risk may be increased by the co-administration of other drugs also known to affect the QTc interval e.g. anti-arrhythmias, anti-histamines.

13.1.3 The presence of disorders affecting metabolism e.g. hypo and hyperthermia, stress and extreme emotions and extreme physical exertion

14. RT IN PREGNANCY

14.1 RT should be avoided if possible during pregnancy. NICE recommendations included in "Antenatal and postnatal mental health" (CG45) should be followed:

14.1.1 When choosing an agent for RT in a pregnant woman, an antipsychotic or a benzodiazepine with a short half-life should be considered

14.1.2 If an antipsychotic is used, it should be at the minimum effective dose because of neonatal extrapyramidal symptoms

14.1.3 If a benzodiazepine is used, the risks of floppy baby syndrome should be considered

14.1.4 During the perinatal period, the woman's care should be managed in close collaboration with a paediatrician, an anaesthetist and obstetrician

15. ASSESSMENT, PRESCRIPTION AND ADMINISTRATION

15.1 **Physical examination** should be made prior to prescribing RT (where reasonably possible), with particular regards to:

- Parkinson's Disease, Lewy body dementia, organic syndromes and acute confusional states
- General condition and weight
- Falls risk, recent falls
- Hydration & nutrition
- Infection
- Evidence of pre-existing cardiac or pulmonary conditions
- Pregnancy
- Baseline pulse, blood pressure, temperature and respiratory rate
- Head injuries and seizures
- Note any illicit drug intoxication, especially alcohol or benzodiazepines, and urine drug screen results where available; the risk of RT may be compounded in people who have recently abused illicit drugs or alcohol
- Baseline electrolytes and ECG where recent test results available. ECG monitoring is advisable and wherever possible a baseline ECG should be done

15.2 **Medical and medication history including**:

- Allergies/ sensitivities/severity of extra-pyramidal side effects (EPSEs)

- Review medication currently prescribed taking note of doses of antipsychotic and benzodiazepines received over the past 48 hours (including the administration of prn medication)
- The benefits and adverse effects of previous medication, and previous history of severe extra-pyramidal side effects
- Previous response to medication or RT and/or other methods of managing imminent violence
- History of any possible illicit drug taking

15.3 **Legal authority** - under which RT is to be given/Advance directives:

- What legal authority is RT being given under?
- Consideration of the Mental Capacity Act/ Mental Health Act must be taken
- Are any advance directives available? However, it is unlikely that in an emergency RT situation, where there are justifiable concerns for safety of the patient or others, that an advanced directive would be relevant in this situation

16. CHOICE OF MEDICATION

16.1 These treatment guidelines are intended to be used for the majority of people. If it is necessary to prescribe treatment not covered by these guidelines then this must first be discussed with an appropriately experienced doctor. See appendix 5 & 7 for prescription guideline specifics

16.2 The individuals' previous response to medication should always be considered when deciding on the choice of drug or dose to use during RT

16.3 Patients at the end of life who are already receiving large doses of sedating drugs (usually midazolam, opiates, and olanzapine) will require different dosages and possibly different drugs for RT. Advice should be sought from a senior palliative care physician / nurse

16.4 The Rapid Tranquilisation order set and RT flowchart should be activated in EPIC (Appendix 1, 2 & 3)

17. REQUIRED MONITORING POST-ADMINISTRATION OF RT

17.1 An RT flowsheet should be activated following an RT event, this will guide nursing care post procedure to keep the patient safe from possible deterioration (Appendix 2 & 3)

17.2 If a patient is unconscious or not rousable, and particularly if there are concerns about respiratory rate, a Doctor / arrest team Ext 2222 should be called immediately and flumazenil made available (Appendix 6)

17.3 High stress levels, hyper-aroused physical state, restraint, agitation and hypokalaemia can all place the person at high risk of developing cardiac arrhythmias, in addition to other known adverse effects associated with prescribing medication indicated for RT

17.4 A heavily sedated or unconscious patient should have a nurse with them at all times until there are no concerns about their ability to maintain their own airway

17.5 To maintain staff safety, even if someone appears sedated or asleep, it is important to keep a safe distance until you can assess that person's mental state on their wakening

17.6 Once alert and responsive, continue to monitor alertness, mental state and behaviour. Re-start physical observations when safe to do so, although assessment of respiratory rate should always be undertaken, even if at a distance

17.7 In certain situations, the risk of provoking further violent behaviour/ assault by imposing physical observations will need to be balanced against the risks of not carrying out these checks. If it is not possible to carry out regular physical observations, due to the person's level of disturbance, this must be clearly stated in the clinical notes, although every effort should be made to observe respiratory rate. Then regular (ie 15 minute) visual observations should be maintained and recorded, until physical observations become possible when it is safe to do so

17.8 Patients who have been heavily sedated are at high risk of developing a pressure ulcer due to their lack of mobility. For this reason, the patient should have increased levels of pressure area monitoring while they are sedated and in the immediate days following

17.9 Staff may be reluctant to disturb a patient who has been previously aggressive, who is now sedated and settled. Once it has been established the patient is no longer a risk to staff when providing care, it is vital that the patient receives all their normal activities of daily living in addition to the pressure area care. Through orientation and hydration the patient will improve more quickly. For example:

- Food and drinks should be offered regularly once the patient is safe to swallow
- Help with hygiene and toileting should be given at regular intervals
- An emphasis on orientating the patient at every interaction should be made
- Usual pressure area care should be re-started once it is safe to do so and staff are no longer at personal risk

17.10 Where a patient is unable to swallow safely due to their recent RT sedation the use of intravenous or subcutaneous fluids should be considered. It would be anticipated that this should be a transient problem, but if a poor swallow persists then a referral to the speech and language department should be made

18. POST INCIDENT REVIEW AND MONITORING REQUIRED AFTER RT

18.1 An RT flowsheet must be activated in EPIC to guide safe monitoring and care post RT

18.2 An incident report must be completed on DATIX following each RT event

18.3 A full and contemporaneous account of the events must be documented in EPIC by the clinicians involved e.g. events leading up to the reason for RT, any restrain required, the administration event and the post recovery period.

18.4 Plans of care should be updated to reflect future plans for RT in light of response to current episode

18.5 Regularly prescribed medication should be reviewed: start, restart or change medication where clinically indicated. Review any 'when required' medication, particularly if they include sedation or opioids

18.6 Once the patient is calmer, an ECG following should be performed when:

- High doses are used
- When used in combination with any other medication known to prolong the QTc interval (click here)
- If the baseline ECG is abnormal or a baseline ECG wasn't available
- If there is any clinical indication to repeat the ECG

18.7 Check electrolytes if not taken in the last 72 hours

18.8 Following any intervention for the acute management of disturbed behaviour, the individual involved must be given the opportunity to talk to a member of staff about the incident. Every effort must be made to establish whether the individual understands why this happened. This must be documented in the individual's notes

18.9 Team de-briefs should be encouraged post incident to help support staff and to ensure there is adequate learning from the incident. Help can be sought from psychiatric liaison teams and occupational health depts

19. LEGAL IMPLICATIONS FOR RT

19.1 It is extremely unlikely that a patient who is distressed enough to require treatment with RT will be in a position to give capacitated consent to this. Therefore, this intervention will need to be given within a legal framework with appropriate safeguards

19.2 If the person is detained in hospital under a treatment order of the Mental Health Act (e.g. sections 2, 3, 4, 37) and the drugs used for RT are explicitly prescribed and part of the authorised treatment plan, then this would be the appropriate legal authority. This would not apply to patients held under sections 5(2) and 5(4) which are temporary detention orders only and do not allow for treatment

19.3 For these patients and patients not subject to the Mental Health Act, the Mental Capacity Act does allow for use of sedation and restraint; however, considerations need to be given to:

- The reasons why the individual is felt to lack capacity to make decisions relating to their care and treatment needs to be clearly documented
- A person is unable to make a decision if they cannot do any **ONE** of the following four things:
  - Understand the information relevant to the decision
  - Retain the information long enough to make a decision
  - Use or weigh up that information as part of the process of making the decision
  - Communicate their decision, whether by talking, using sign language or any other means
- The reasons why this course of action is proportionate to the risks faced and in the patient's best interests needs to be documented (including proposed risks and benefits of RT versus risks and benefits of not administering RT)

19.4 Documentation need not be lengthy and should not delay emergency treatment; however, an assessment of capacity is mandatory and a note should be created in EPIC once the patient has been treated and situation is calm

19.5 If RT is administered under the Mental Capacity Act, it is likely that this individual is being deprived of their liberty in some way. It is necessary to consider what legal framework is employed following discussion. This may be DoLS or MHA. The MCA teams in East and North can be contacted for advice

20. MONITORING COMPLIANCE WITH THIS GUIDELINE

20.1 A Datix report must be completed for each episode where RT is required. Divisional governance teams are expected to act on the Datix and inform divisional governance meetings about trends and work undertaken in response to Datix reporting

20.2 RT flowsheets activates a rapid Tranquilisation patient list in EPIC. This list is monitored by the MCA team and the dementia & delirium nurse specialists who will report their findings to their own governance groups

20.3 These incidents will be reviewed and audited quarterly by the Restrictive Practice Steering Group

21. ASSOCIATED CLINICAL GUIDELINES OR POLICIES/PROCEDURES

- Clinical Guideline for the Diagnosis and Management of Delirium.
- Confusion, Delirium and Dementia care plan
- Falls Reduction and Management Policy
- Mental Capacity Act guidelines.
- Mental Capacity Act Code of Practice
- DOLs policy and guidelines.
- Safeguarding policy and referral guidelines.
- Risk Assessment Policy and Procedure.
- Incident reporting, analysing, investigating and learning policy and procedures.
- Management of Violence, Aggression and Challenging Behaviour Policy.
- Alcohol and Substance Misuse Policy
- NICE Guideline CG103, Delirium: prevention, diagnosis and management in hospital and long-term care
- NICE Guideline NG10, Violence and aggression: short-term management in mental health, health and community settings
- NICE Guideline [CG45], Antenatal and postnatal mental health: Clinical management and service guidance
- NICE Guidelines [CG11], Challenging behaviour and learning disabilities: prevention and interventions for people with learning disabilities whose behaviour challenges
- NICE guidelines [CG42] Dementia: supporting people with dementia and their carers in health and social care
- Learning Disabilities Care Plan
- Mental Capacity Assessment Form

22. ARCHIVING ARRANGEMENTS

The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely

23. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY/ STRATEGY

To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
RT events activated on EPIC	EPIC patient list Database kept by MCA team	Divisional Governance groups
RT Datix events	Datix	Divisional governance groups

APPENDIX 1: EPIC RAPID TRANQUILISATION (RT) FLOWSHEET

Flowsheets

File | Cosign | Add Rows | LDA Avatar | Add Col | Insert Col | Data Validate | Hide Device Data | Last Filed | Reg Doc | Graph | Go to Date | Responsible | More

Vitals | I/O | Adult Screenings | IV Assessment | Nursing Needs | Wound Assess/Care | Moving and Handling | Patient Belongings | Rapid Tranquilisation... | Rapid Tranquilisati...

Search (Alt+Co... | Accordion | Expanded | View All | 1m 5m 10m 15m 30m 1h 2h 4h 8h 24h | Interval Start: 07:00 | Reset | Now

Hide All | Show All

General and su... | Rapid Tranquill... | Rapid tranquili... | Confirm medic... | POST RT Moni... | POST RT at 30... | Vital Signs | Security Called

	Admission (Current) from 11/5/2023 in W...			
	16/5/2023 20:22:11	17/5/2023 09:00		Last Filed
General and supportive measures/ de-escalation				
Environmental factors				
Physical				
Emotional				
Rapid Tranquillisation Review/ Before Pharmacological treatment				
Does your patient have acutely disturbed/ challenging behaviour?				
Time behaviour escalated				
Have you attempted to de-escalate bahaviour using general and supportive measures?				
Patient family/ carer informed and asked if they are able to assist?				
Do you have a 1:1 available for this patient?				
Rapid tranquilisation review/ post pharmacological treatment				
1:1 Nursing arranged				
Senior nurse and site managment team aware				
Incident recorded on datix				
Confirm medication administered for rapid tranquilisation				
Rapid Tranquilisation: Please confirm this medication is being administered for RT				
POST RT Monitoring				
Level of Consciousness				
POST RT at 30 mins and then make a plan				
Pressure Ulcer prevention				
Vital Signs				
Resp	17			17
SpO2	97			97 %
SpO2 Probe choice				
Oxygen Therapy	None (Room air)			None (Room air)
O2 Flow Rate (L/min)				0 L/min
FiO2 (%) (O2 flow rate %)				
O2 Delivery Method				
Airway Adjuncts				
BP	169/86			169/86
BP Location				
BP Method				
Patient Position				Lying

APPENDIX 2: EPIC RAPID TRANQUILISATION ORDER SETS

Rapid Tranquillisation

Accept

TO BE USED AFTER NON-PHARMACOLOGICAL ATTEMPTS: E.G DISTRACTION, DE-ESCALATION HAVE FAILED AND AFTER ORAL MEDICATION IS INEFFECTIVE OR HAS BEEN REFUSED.

WHEN PRESCRIBING RT DRUGS, PLEASE TAKE INTO ACCOUNT ANY ORAL DOSES THAT MAY HAVE BEEN GIVEN WITHIN THE LAST 24 HOURS.

- RD&E Guideline for Rapid Tranquilisation of Adults

- Rapid tranquilisation in physically frail adults or those with dementia
- Rapid tranquillisation in adults (non-frail/no dementia)

Next Required

Accept

Rapid Tranquillisation

Accept

TO BE USED AFTER NON-PHARMACOLOGICAL ATTEMPTS: E.G DISTRACTION, DE-ESCALATION HAVE FAILED AND AFTER ORAL MEDICATION IS INEFFECTIVE OR HAS BEEN REFUSED.

WHEN PRESCRIBING RT DRUGS, PLEASE TAKE INTO ACCOUNT ANY ORAL DOSES THAT MAY HAVE BEEN GIVEN WITHIN THE LAST 24 HOURS.

- RD&E Guideline for Rapid Tranquilisation of Adults

- Rapid tranquilisation in physically frail adults or those with dementia

Alternative options for physically frail adults or those with dementia

It may be necessary to consider the following treatment options - this must first be discussed with "an appropriately experienced doctor":

- Repeat Lorazepam IM & consider use of 'adult' doses (caution with accumulation)
- Consider Olanzapine 2.5- 5mg IM
- Aripiprazole may be considered only with psychiatric advice

Consider promethazine IM if lorazepam contra-indicated or for benzodiazepine tolerant person only on advice of consultant psychiatrist

- 1st Line - lorazepam
- 1st Line (lorazepam shortage) - midazolam
- 2nd line - olanzapine
- Rapid tranquillisation in adults (non-frail/no dementia)

Next Required

Accept

Rapid Tranquillisation

Accept

TO BE USED AFTER NON-PHARMACOLOGICAL ATTEMPTS: E.G DISTRACTION, DE-ESCALATION HAVE FAILED AND AFTER ORAL MEDICATION IS INEFFECTIVE OR HAS BEEN REFUSED.

WHEN PRESCRIBING RT DRUGS, PLEASE TAKE INTO ACCOUNT ANY ORAL DOSES THAT MAY HAVE BEEN GIVEN WITHIN THE LAST 24 HOURS.

- RD&E Guideline for Rapid Tranquilisation of Adults

- ○ Rapid tranquilisation in physically frail adults or those with dementia
- ◉ Rapid tranquillisation in adults (non-frail/no dementia)
  - ○ 1st line - lorazepam
  - ○ 2nd Line - olanzapine
  - ○ 2nd Line (Lorazepam shortage) - midazolam

Next Required

Accept

APPENDIX 3: EPIC RT REVIEW AND WORK LIST ACTIVATION

New Orders

Rapid Tranquilisation Review

Until discontinued, Starting today at 11:28, Until Specified

Work List

Add Task | Today 06:00 - 18:00 | Only Overdue | Filter: Flowsheets

Time

Time	Type	Task	Frequency	Priority
11:21		**Continuous sp02 monitoring**	Continuous x 4 hours	Routine
11:21		**Post RT Monitroing every 15 minutes**	q15 min	Routine
11:21		**Post Rapid tranqilisation every 30 minutes for first hour**	q30 min	Routine
11:21		**Post Rapid tranquillisation monitoring every 30 mins**	q30 min	Routine
11:36		**Post RT Monitroing every 15 minutes**	q15 min	Routine
11:51		**Post RT Monitroing every 15 minutes**	q15 min	Routine
11:51		**Post Rapid tranqilisation every 30 minutes for first hour**	q30 min	Routine
11:51		**Post Rapid tranquillisation monitoring every 30 mins**	q30 min	Routine

12:00

APPENDIX 4: MANAGEMENT OF CLINICAL CONCERNS DURING RT

Problem	Remedial Measures
Acute dystonias (including oculogyric crisis) and parkinsonism (see also below)	Procyclidine 5 – 10 mg IM (repeat after 20 minutes if necessary) or IV (repeat after 10 minutes if necessary) Older adults: Procyclidine 2.5 – 5mg
Increased temperature or marked muscular rigidity (risk of Neuroleptic Malignant Syndrome and perhaps arrhythmias)	▪ Withhold antipsychotics ▪ Monitor closely - send bloods for CPK level ▪ Cool the person ▪ Seek urgent medical advice immediately AND refer to ITU if continued or any other signs of NMS, such as sweating, hypertension or fluctuating BP, tachycardia, incontinence/retention/obstruction, muscular rigidity (may be confined to head and neck) or confusion, agitation/altered consciousness.
Respiratory depression: respiratory rate <10/minute after lorazepam or if oxygen saturation <90%)	Oxygen and flumazenil (a benzodiazepine antagonist) may be given if benzodiazepine-induced respiratory depression. <u>There are risks associated with the use of flumazenil and so it should only be used in life threatening situations.</u> See flow chart below in Appendix 6 for how to give Flumazenil. If flumazenil is contraindicated or ineffective mechanical ventilation is likely to be required and urgent anaesthetic advice is necessary.
Irregular/slow (<50/min) pulse	Monitor closely and seek clinical review
Hypotension Fall in blood pressure (Orthostatic or <50mmHg diastolic)	Lie person flat, raise legs if possible Monitor closely

APPENDIX 5: DRUG TREATMENTS FOR RAPID TRANQUILLISATION (RT)

When prescribing RT drugs, take into account any oral doses that may have been given within the last 24 hours.
CAUTION RESPIRATORY DEPRESSION: Where more than 2 doses of lorazepam are required for RT, a doctor should review the individual before a 3rd dose is given. Midazolam is associated with a higher risk of respiratory depression than lorazepam.

Adults 18yrs > (without dementia)			
	Drug	**Usual Dose**	**Guideline Max Dose**
FIRST LINE	**Lorazepam IM***	2mg initially May be repeated after 60 minutes (see respiratory depression caution above)	Rarely, higher doses over 24 hours may be needed up to 0.12mg/ Kg/24hrs for under 65s (Equates to 8mg in a 70kg person)
SECOND LINE	**Olanzapine IM** Do not give within one hour of a benzodiazepine.	10mg initially; 5 to 10mg may be repeated after 2 hours	20mg/24hours (all formulations) No more than 3 injections in 24 hours
THIRD LINE **ONLY ON ADVICE OF CONSULTANT**	**Haloperidol + Promethazine IM** Not recommended for use unless a recent pre-treatment ECG is available and individual has had a previous good response to it.	Haloperidol 5mg + Promethazine 25mg (50mg may be given if clinically indicated) may be repeated after 1 hour.	Haloperidol 12mg + Promethazine 100mg/24 hours.

*Where **Lorazepam IM** is unavailable due to supply problems then give **Olanzapine IM First Line (As above),** consider **Midazolam IM Second Line (As below)** and **IM Haloperidol Third Line (As above)**

SECOND LINE IF LORAZEPAM UNAVAILABLE	**Midazolam IM**	5mg may be repeated after 1 hour (see respiratory depression caution above)	15mg/24 hours

Alternative options for adults 18 years >
The above guidelines are intended to be used for the majority of people, however it is recognised that there are occasions where these options may not be suitable. If it is necessary to prescribe treatment outside these guidelines then this must first be discussed with a consultant liaison psychiatrist or senior physician. It may be necessary to prescribe an alternative treatment or higher dosages in order to effectively treat the individual. Care should be taken with patients at the end of life on high doses of sedation, senior Palliative Care advice is required

Physically Frail Adults 18 years > or those with Dementia of any age			
	Drug	**Usual Dose**	**Guideline Max Dose**
FIRST LINE	**Lorazepam IM****	1mg may be repeated after 60 minutes (caution respiratory depression if more than 2 doses given)	0.06mg/kg/24 hours Equates to?
FIRST LINE IF LORAZEPAM UNAVAILABLE	**Midazolam IM****	2.5mg may be repeated after 60 minutes (see caution above)	7.5mg/24 hours

Alternative options for physically frail adults or those with dementia
It may be necessary to consider the following treatment options - this must first be discussed with an appropriately experienced doctor:

- Repeat Lorazepam IM & consider use of 'adult' doses (caution with accumulation)
- Consider Olanzapine 2.5- 5mg IM
- Aripiprazole may be considered, only with psychiatric advice
- Consider promethazine IM if lorazepam contra-indicated or for benzodiazepine tolerant person only on advice of consultant geriatrician, physician or psychiatrist

APPENDIX 6: HOW TO USE FLUMAZENIL

Flowchart describing the use of IV flumazenil in the event of respiratory depression associated with benzodiazepines given as part of the Rapid Tranquillisation Protocol. It is expected either the arrest team on Ext 2222 or an anaesthetist will be called to the ward to assist and provide senior advice and support.

There are risks associated with the use of flumazenil it should only be used in life threatening situations.

If, after the administration of benzodiazepines, the individual appears to go into respiratory depression and respiration falls below 10 /minute:

Call Cardiac Arrest Team Ext 2222
Give Flumazenil 200micrograms I/V[1] over 15 seconds

↓

If required level of consciousness is not obtained, after 60 seconds, give another Flumazenil 200micrograms I/V[1] over 15 seconds

↓

Repeat process until consciousness is achieved (maximum of 6 cycles, i.e. a maximum dose of flumazenil of 1mg/24 hours)

[1]To be given via cannula, into a large peripheral vein to minimise pain on administration

Notes

- This is only part of the initial stage of managing respiratory depression induced by benzodiazepines.
- Monitoring respiration rate and the use of a pulse oximeter will help to assess whether Flumazenil should be used. Consider using oxygen and providing assisted ventilations via Bag-Valve-Mask the individual if required
- The duration of action of flumazenil (half-life 52 minutes, duration of action 1-2 hours) is much shorter than the benzodiazepines; therefore, the respiratory function may recover and then deteriorate again.
- Flumazenil may precipitate:
  - a withdrawal syndrome in benzodiazepine-dependent individuals
  - convulsions in epileptics
  - arrhythmias in people who have taken cardiotoxic drugs.
- Do **NOT** give flumazenil for mixed overdoses particularly combined tricyclic antidepressant and benzodiazepine overdosage as convulsions and cardiac arrest may be precipitated.
- Flumazenil should be used with caution in people with a history of seizures, head injury, hepatic impairment or chronic benzodiazepine use.
- **MONITORING** (recommended for at least 4 hours) – Heart Rate, Respiratory Rate, BP and alertness

APPENDIX 7: DRUG INFORMATION FOR DRUGS INCLUDED IN RT POLICY

Drug	Place in Treatment	Administration	Cautions and Additional Comments
IM Lorazepam	1st Line Benzodiazepines are relatively safe and their effects can be reversed with the antagonist flumazenil.	Dose 1-2mg (depending on age and if dementia present) Up to 0.12mg/kg/24hrs Lorazepam injection (stored in the fridge) should be diluted 1:1 with Water for Injection immediately before IM use.	Risks with benzodiazepines include loss of consciousness and respiratory depression or arrest. Flumazenil injection (short acting benzodiazepine antagonist) must be available for benzodiazepine-induced respiratory depression. The duration of action of lorazepam is greater than flumazenil so repeated doses of flumazenil may be necessary if there is recurrence of respiratory depression. The pharmacokinetics of lorazepam are the same whether given orally or IM. The only reason to give IM lorazepam is if the person refuses oral. Concern is sometimes expressed about the possibility of disinhibition following the administration of benzodiazepines. There is conflicting evidence available about this. It is, however, a rare side effect. Disinhibition maybe more likely to occur in those with organic brain disease, including learning disabilities, the under 18s and the over 65s, and perhaps those with impulse control problems.
IM Olanzapine	2nd Line Where benzodiazepines are not considered to be appropriate, such as for individuals who are tolerant to benzodiazepines or where benzodiazepines have been ineffective in the past. It should be noted that antipsychotics used during rapid tranquillisation are used for their sedative rather than antipsychotic properties.	Dose: 5-10mg (usual dose 10mg) followed by 5-10mg after 2 hours if necessary Up to 20mg in 24 hours (maximum combined oral and IM) Maximum 3 injections in 24 hours for 3 days	IM olanzapine has been shown in a number of studies to be at least as effective and overall more effective than IM haloperidol with significantly less extrapyramidal side effects or risk of QT prolongation and may have a faster onset of action. **If the person is considered to need IM benzodiazepine treatment, this should not be given until at least one hour after IM olanzapine administration.** If the person has received IM benzodiazepine, IM olanzapine administration should only be considered after careful evaluation of clinical status, and the person should be closely monitored for excessive sedation and cardiorespiratory depression.

Drug	Place in Treatment	Administration	Cautions and Additional Comments
			IM olanzapine results in a sharp, high peak concentration then a rapid fall; oral olanzapine takes a longer time to reach a lower peak concentration and takes longer to clear.
IM Haloperidol	3rd Line It is recommended that this is not used unless a pre-treatment ECG is available **and** the person has had a previous history of a good response to haloperidol and has tolerated it well. There may however be situations where other treatment options have failed and based on an assessment of the risks/benefits to the individual haloperidol IM is given despite these recommendations not being met – the reason for proceeding with haloperidol in these circumstances should be clearly documented in the individual's notes.	Dose: 2-5mg Up to a maximum of 12mg in 24 hours.	IM haloperidol is no longer recommended first line for rapid tranquillisation due to: • High incidence of acute dystonia • Requirement for a pre-treatment ECG Haloperidol and lorazepam IM should no longer be used routinely in combination. The data sheet for lorazepam IM states that there have been reports of apnoea, coma, bradycardia, heart arrest and death with the concomitant use of lorazepam injection solution and haloperidol. **Procyclidine injection must be available**. It is recommended that it is prescribed with IM haloperidol to reduce the risk of dystonia/other EPSEs. The bioavailability of IM haloperidol is 50% greater that oral haloperidol. Prescriptions for oral and IM medication must be written separately (not PO/IM).
Midazolam	2nd Line only when Lorazepam is unavailable.	In reported trials, the dose used has ranged from 2.5mg to 15mg, although it must be noted that the higher doses appear to induce sleep, whereas the aim of rapid tranquilisation is to promote a calmer state and **not** induce sleep/unconsciousness.	Like other benzodiazepines midazolam has the potential to cause respiratory depression but this risk is thought to be greater than with lorazepam. Always ensure that emergency resuscitation equipment and medication (including flumazenil) is present before administration. **Remember midazolam acts for a longer period than flumazenil - additional doses of flumazenil may be required.** Following IM Midazolam, anterograde amnesia of short duration may occur with the individual not remembering events that occurred during the maximum activity of the compound.

Drug	Place in Treatment	Administration	Cautions and Additional Comments
Midazolam cont..		The recommended dose is 5mg for adults over 16 years and 1 to 2.5mg for physically frail adults. **Only the 10mg/5ml strength should be stocked for rapid tranquillisation on inpatient wards**:	Midazolam is now a schedule 3 controlled drug, therefore must be ordered from the pharmacy department using the ward controlled drug requisition book in accordance with Trust standard operating procedure CD1. Once received, midazolam injection should be recorded in the controlled drugs register and stored in the controlled drugs cupboard. The NPSA issued an alert on reducing risk of overdose with midazolam injection in adults. Link:http://www.nrls.npsa.nhs.uk/resources/?entryid45=59896. The strength of midazolam injection recommended in this Guideline is classed by the NPSA as being high strength. However the volume required of the lower strength preparation to allow the doses needed for rapid tranquillisation would be too large. To minimise the risk of errors in dosing occurring **only the 10mg/5ml strength should be stocked for rapid tranquillisation on inpatient wards**. If in exceptional circumstances there is a clinical need for a different strength of midazolam injection to be kept on a ward then an individual risk assessment should be done and all possible steps taken to avoid any confusion between the different strengths.

APPENDIX 8: STRATEGIES TO MANAGE CHALLENGING BEHAVIOUR

OBSERVING RESTLESSNESS	
INTERVENTION	**RATIONALE**
Learn to recognise a problem situation and intervene before it becomes a crisis	Being aware of the person's increased agitation, this may allow you to defuse the situation before it gets out of control
Try to keep the patient away from situations and people that provoke them	Aggressive behaviour does not happen in a vacuum, be aware of the environment, and try to identify what may have triggered the behaviour
Use a gentle voice and be reassuring	Being calm may help to defuse the situation
Approach slowly and calmly in front, talk to the person, tell them what you are going to do	Sudden movements and coming up behind the person may be seen as threatening
Use touch if appropriate and you know the patient well, bear in mind touch may provoke	Touch can be reassuring if appropriate and timely
Use non-threatening postures, sit down so that you are at eye level, do not stand over the person	Important to build trust with your patient
Ask the person about the problem, gradually turn attention to something more pleasant and unrelated, change activities, go to another room	The person feels that they are being listened to
Establish a calm environment	You will need to give the person time to calm down
Avoid arguing or trying to reason, back off and avoid confrontation whenever possible	Arguing and reasoning may increase the persons feelings of distress, confrontation may escalate into a violent incident
If you are unable to cope with the situation ask a colleague to take over	If you are becoming stressed the patient will become aware of your verbal and non-verbal cues and respond accordingly
If the person is refusing to comply with your request, providing they are in a safe place, back off and return later	The patient may be unable to tell you that they do not want you to carry out an intervention at this time, may be more compliant later
Avoid physical restraints	Restraints increase the persons perception of threat
Keep a record of behaviours using the Behaviour Chart Flowsheet on EPIC	To help monitor fluctuations in behaviour and assess possible triggers

POTENTIAL VIOLENCE DIRECTED AT OTHERS	
INTERVENTION	**RATIONALE**
Listen actively to the patient Remain honest calm, clear and concise when talking Ensure appropriate eye contact and facial expression	Establishes a sense of trust Conveys attention Facilitates communication
Do not rush the patient or structure the environment	Tension can develop if the patient feels forced or pushed into acting in a certain way
Remove all potentially harmful items from the patients reach, glasses, bottles etc	Objects may be used by the patient to injure others or themselves
Reinforce non – violent behaviour	Rewarding positive behaviour discourages negative outbursts
Encourage the patient to do as much as possible for him/herself	Invading the patient's personal space may cause physical aggression
Promote regular exercise	Helps control and channel the patient's energies
Avoid arguing to trying to reason with the patient	The patient cannot appreciate another point of view at such times
Do not use aggression or make threats when dealing with patients	A calm approach is important in reducing the potential for aggression
If you feel the situation may get out of control, call for assistance from other ward staff and security. Do not allow yourself to be blocked in a room by the patient	Keeping the patient, other staff, patients and visitors and yourself is of prime importance. These situations can escalate quickly. It is important to have thought about calling others in to assist and support you, often from afar at first to avoid increasing potential conflict

APPENDIX 9: COMMUNICATION PLAN

COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the policy	All clinical staff and senior management
The key changes if a revised policy	• Merged document with Eastern guideline and Northern policy • Streamlined document, with minor updates • Inclusion of new flowchart • Addition of EPIC documentation
The key objectives	• Inform staff of their responsibilities in relation to the safe administration and after care of rapid tranquilisation • Minimise the risk of patient harm. • Set out the Trust's responsibilities for monitoring and acting on Trust wide learning from RT events
How new staff will be made aware of the policy and manager action	• Email to senior staff to cascade to their staff to help ensure existing staff are informed of the changes • Training sessions • HUB updates • Comm cells
Specific Issues to be raised with staff	• Their training requirements • Care needs of an acutely disturbed and potentially dangerous patient who lacks capacity • RT should be a rare event, other measures should be considered to help reduce the patient's distress • Legal authority to use RT
Training available to staff	• Trust's RT training – under review with TN by Lisa Denning team • E-learning • Psychiatric liaison teams
Any other requirements	
Issues following Equality Impact Assessment (if any)	
Location of hard / electronic copy of the document etc.	The original of this policy will remain with the author. An electronic copy will be maintained on the Trust Intranet Hub. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely.

APPENDIX 10: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	Rapid tranquilisation (RT) policy – inpatient acute ward based
Division/Directorate and service area	Trust wide
Name, job title and contact details of person completing the assessment	Debbie Cheeseman Consultant nurse for older adults
Date completed:	07.09.23

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

This policy is intended to protect people from harm and to provide practice standards to all staff who are involved in the prescription or administration of intramuscular (IM) Rapid Tranquilisation (RT) medication for the control of disturbed behaviour

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☐ Staff ☒ Patients ☒ Other (please specify)

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☒	☐
Disability	☒	☐
Sex - *including: Transgender, and Pregnancy / Maternity*	☒	☐
Race	☒	☐
Religion / belief	☒	☐
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒

4. Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to… (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

Rapid tranquilisation is only advocated as the very last option when all other treatment pathways have been exhausted. This policy is for all adults 18yrs>, but it is accepted that some patient groups may be at increased risk of having Rapid Tranquilisation;

- Patients with dementia are more at risk of having a Rapid Tranquilisation event due to the increased likelihood of acute delirium during a hospital stay which can result in verbal and physically aggressive behaviours
- Patients who are de-toxing from drug and alcohol abuse are at greater risk of challenging behaviours which may cause harm to themselves and others and are frequently subject to Rapid Tranquilisation to enable medical treatment to be administered
- Evidence suggests that black and ethnic minorities are at increased risk of restraint. There is the potential for discrimination in relation to the use of rapid tranquilisation on grounds of age, male gender and ethnicity (for example young black men). This could be due to conscious or unconscious bias or due to genuine societal issues.
- Language and cultural difference may have a negative impact on this patient group feeling heard and understood. This may lead to increased anxiety, which might impact on their behaviour and result in an increased chance of being subjected to Rapid Tranquilisation
- Consideration should be given to people who may have communication difficulties e.g. autism, learning disabilities, dementia, speech & hearing difficulties, cultural and language differences
- The policy is applicable to both Mental Health and Learning Disabilities Services patients who are exhibiting harmful behaviour that fails to be controlled by all other interventions. Whilst not all who exhibit violence and aggression of this nature are mentally ill or have a learning disability, there is a significantly increased likelihood of this behaviour (and its failure to be controlled through other approaches) in this group. However, this concern is justified in the context of protecting from harm both the service user and those in their surroundings (staff and other service users, visitors etc).
- Evidence suggests more males are subject to Rapid tranquilisation and therefore may be more justifiably affected by this policy.
- Wider evidence suggests that there is a greater prevalence of mental health conditions in individuals who are trans or gender dysphoric than in the general population. Should rapid tranquilisation be required for a trans/gender dysphoric patient, dosage will be determined on factors relating to assessment of the individual and any other medications they may be taking.
- It may be necessary to be sensitive to patients who fast for religious reasons (e.g. Muslim patients observing Ramadan may prefer to take medications during non-daylight hours. A decision would have to be made in relation to application of rapid tranquilisation as to a balance between clinical need and safety and the person's religious observance. Additionally, there is a potential for bias in application of rapid tranquilisation for some religious groups, although no evidence is known to exist in relation to this.

5. Do you think the document meets our human rights obligations? ☒

Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:
• ***Fairness –*** assessing the patient as an individual is paramount to this policy. Assessing the legal authority to act is an essential component. • ***Respect –*** Consideration of the patient's needs, a full assessment of their clinical requirements to ensure actions are taken in their best interest • ***Equality –*** Individual assessment is stressed throughout the document based on clinical symptoms. • ***Dignity –***Consideration given to the need for privacy and use of separate room where possible to maintain dignity but also to use to de-escalate the need for RT • ***Autonomy –*** Stresses importance of legal authority to act

6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?

• Wide consultation across Northern and Eastern sites with clinical and governance teams • Consultation with Devon Partnership Trust • This policy will have a positive impact for three protected characteristics of 'age, disability and race'. Having a more robust policy will help to reduce the potential negative impact on these groups that an RT event may cause

7. If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact please note this below and how this will be monitored/addressed.

"Protected characteristic":	**N/A**
Issue:	
How is this going to be monitored/ addressed in the future:	
Group that will be responsible for ensuring this carried out:	